Overdose / Toxic Ingestion

TE-07 | Toxicology & Environmental

Quick flow

01 ABCs + Exposure History [EMT / AEMT / Paramedic]

Substance, route, amount, time, co-ingestants, intent; bring containers when safe. For skin/eye exposures, remove contaminated clothing and irrigate; for inhalation, move to fresh air only when the scene is safe.

02 Inadequate Ventilation? [EMT / AEMT / Paramedic]

Ventilate first; suspected opioid: naloxone titrated to breathing.

03 Shock / AMS / Seizure? [EMT / AEMT / Paramedic]

Treat by age-appropriate shock, diabetic, AMS, behavioral, and seizure pathways.

04 Poison Control [EMT / AEMT / Paramedic]

Call 1-800-222-1222; obtain 12-lead, IV/IO, and cardiac monitor as indicated.

05 Specific Toxin? [EMT / AEMT / Paramedic]

APAP/salicylate • beta/CCB • CO/cyanide • organophosphate • TCA. Use the current medication reference and Poison Help/Medical Control; do not use unapproved antidote doses.

06 Treat + Disposition [EMT / AEMT / Paramedic]

Patients treated with naloxone require healthcare observation for recurrent respiratory depression. Other low-risk exposures may follow Poison Help home-care advice only when agency non-transport criteria are met. Intentional, pediatric, pregnant, polysubstance, or symptomatic exposures require direct Poison Help consultation.

Overview

- Stabilize ventilation and perfusion first, identify time/substance/amount/co-ingestants, and consult Poison Control.
- Naloxone is titrated to adequate ventilation - not restoration of consciousness - and toxin-specific treatment follows the clinical toxidrome and ECG.

Provider-Level Actions

Protect rescuers, stop exposure when safe, support ABCs, and involve Poison Control or Medical Control early.

EMT

- Support ventilation, administer naloxone for suspected opioid-induced respiratory depression only per the current Medication Reference, check glucose, and contact Poison Help.
- If activated charcoal is stocked and authorized, use only after Poison Control or Medical Control recommendation and only when airway protection and swallowing are intact.

AEMT

- Perform all EMT care plus IV/IO access, broader-route naloxone per the current Medication Reference, reassessed fluids, and toxin-specific treatment only with Poison Help or Medical Control guidance.

Paramedic

- Perform all prior care plus cardiac monitoring, pacing when indicated, and toxidrome-specific advanced care using the current Medication Reference with Poison Help or Medical Control guidance.

Indications

- Known or suspected overdose, poisoning, or toxic ingestion/exposure.

Warnings / precautions

- Ventilation has priority over naloxone; do not dose solely to restore consciousness. Use the Medication Reference for approved naloxone dosing.
- Charcoal is not routine and should not be given to a patient unable to protect the airway; use only when stocked, authorized, and recommended by Poison Control or Medical Control.
- Do not clear a patient only because they awaken after naloxone; recurrent respiratory depression can occur and requires healthcare observation.
- Do not rely only on history in a suspected intentional ingestion; assess for hidden medications and weapons.

Related pathways / contacts

Call Poison Help - National Poison Control line

Medication Reference - Open for approved naloxone and antidote dosing.

TE-08 WMD / Nerve Agent - Open for scene safety, decontamination, recognition, and escalation of suspected cholinergic exposure.

Source and references

Claiborne County EMS approved protocol manual. | Poison Help national line: 1-800-222-1222. | Current Tennessee EMS scope of practice and Claiborne County standing orders control for all provider permissions.

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